جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : رشا رمضان احمد علام
المحافظة : الجيزه **الحي / المدينة :** الوراق **العنوان :** 0
رقم البطاقة : 27907250102485 **رقم التليفون :** 01123072403 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 25-07-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رشا رمضان احمد علام
National ID: 27907250102485

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

Patient Name	رشا رمضان احمد	**Case No.**	**384856**
Sex/Age	**Female 47 Y**	**Patient No.**	**28993**
Registered Date	**15/9/2026 02:25:45 PM**	**Referred By Dr.**	**Surgery**
Validation Date	**10/10/2026 10:14:25 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**17/10/2026 03:03:17 PM**	**Pathology No.**	**2970/26**

Pathology Report

Clinical Data: Right breast cancer, post-NACT.

Nature of the specimen: Right modified radical mastectomy.

Gross Examination:

* Right modified radical mastectomy specimen measuring 25x10x6 cm, with covering skin ellipse measuring 25x10 cm. Nipple & areola looked unremarkable grossly. Serialling showed a mass measuring 5.5x5x5cm, reaching the skin grossly with no evidence of ulceration. The mass located 6 cm from nipple & areola & was 0.2 cm away from the least deep muscular margin.

* **Axillary lymph nodes:** Axillary fat measuring 6x6x3 cm, entangling 11 lymph nodes, the largest measured 2x1.5x0.5 cm.

* **Block designation (10 Blocks):**

1,2,3,4- Mass., 5- Mass & deep margin., 6- Mass & skin., 7- Nipple & areola., 8,9,10- Lymph nodes.

Gross was done by: Dr. Aya Ezzat.

Microscopic examination:

- **Histological type:** Residual viable necrotizing invasive duct carcinoma (cellularity 70%).
- **Grade:** 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- **Tumor size:** 5.5 x 5cm.
- **Tumor Cellularity:** 70%.
- **Ductal carcinoma in situ:** 2% Intermediate grade ductal carcinoma in situ (solid & cribriform pattern with comedo type necrosis).
- **Angioinvasion:** Present.
- **Microcalcification:** Present.
- **Perineural invasion:** Present.
- **Skin:** Tumor infiltrates superficial dermis & colonize epidermal covering with no evidence of ulceration.

GUSTAVE ROUSSY INTERNATIONAL EGYPT
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	رشا رمضان احمد	**Case No.**	**384856**
Sex/Age	**Female 47 Y**	**Patient No.**	**28993**
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- **Axillary lymph nodes:** Positive viable metastatic deposits in two out of eleven axillary lymph nodes (2/11) (The larger metastatic focus measuring 1.4 cm, with extra-nodal extension).
- **Resection Margins:** Free deep margin (Free muscle).
- **Response to therapy:** Pathological partial response to therapy.

Diagnosis

Right breast (Post NACT), Right modified radical mastectomy.

- Histological type: Residual viable necrotizing invasive duct carcinoma grade 3 (Cellularity 70%) (Actual tumor size 5.5x5 cm).

- Tumor infiltrates the superficial dermis and colonize epidermal covering with no evidence of ulceration.

- Ductal carcinoma in situ: 2% Intermediate grade ductal carcinoma in situ (solid & cribriform patten with comedo type necrosis).

- Deep margin: Free.

- Axillary lymph nodes: Positive viable metastatic deposits in two out of eleven axillary lymph nodes (2/11) (The larger metastatic focus measuring 1.4 cm, with extra-nodal extension).

- Stage: ypT3, ypN1a.

- Response to therapy: Pathological partial response to therapy.

- Residual cancer burden: 4.297, RCB class-III.

Signature

Prof. Dr. Maha Guimei, MD, PhD.

Dr. Duaa M. Ali, FEBP.

Pat. Name: Rasha Ramadan Ahmed	Pat. ID: 3505736
Procedure Name: MRI Liver	Date: 21/02/2026



Technique of examination:

- Axial T1 & T2 WIs "Plate 1 & 2".
- Coronal T1 & T2 WIs "Plate 3 & 4".
- Post contrast axial and coronal T1 WIs "Plates 5,6".

MRI Findings:

- Right breast mass lesion 7x3.2 cm
- Mild hepatomegaly showing left lobe segment IV focal lesion 13 mm presenting low T1 , high T2 signal , displaying progressive nodular contrast enhancement in arterial and portal phases completely illed with contrast in delayed phase . No dilatation of the intra hepatic-biliary radicals.
- Normal appearance of the spleen, pancreas, aorta, IVC and both kidneys.
- No ascites seen.
- No para aortic lymphadenopathy noted.
- Normal MRI appearance of the lung bases.

Conclusion:

- ***Right breast neoplastic mass lesion***
- ***Mild hepatomegaly showing left lobe segment IV focal lesion likely representing hemangioma***

Best regards,,
Dr. Sherif Aly, MD

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

M.R.I
[[DOCTYPE:MRI]]

[illegible]

[illegible]

[illegible]

[illegible]

MDT FORM

[illegible]	[illegible]
Age:	[illegible]
Demographic Status:	Phone:
KRS SCORE:	Marital Status:
BMI:	Family [illegible]
CO Morbidity:	Reason of MDT:

Clinical Data:

EX: R L

ER PR HER2 KI67

Pathological Summary:

[illegible]



TNM

Key Words:

[illegible]

History:

Recommendations:

MDT Outcome & Plan

[illegible]

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY
INTERNATIONAL ACADEMY

Patient Name	: الأستاذه / رشا رمضان احمد	Request No.	: 13004531
Patient No.	: 27907250102485	Request Date	: 17/01/2026
Path No.	: 127/26	Verification Date	: 21/01/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass, BIRADs 5.
Nature of Specimen: Tru-cut biopsy, collected on 17/01/2025, at 2:20 pm, cold ischemia time: 20 seconds.

Gross Examination:
Three grey white tissue cores, the longest is 1.5 cm. Totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* DCIS: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.

* IPT (Positive control showing appropriate immunoreactivity):
- ER: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 40% of tumor cells (score 7/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 70%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 3.
IPT:
- ER: Positive (score 7/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Ki67%: 70%.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report [[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL AGUAH

Patient Name	: الأستاذه / رشا رمضان احمد	Request No.	: 13004531
Patient No.	: 27907250102485	Request Date	: 17/01/2026
Path No.	: 127/26	Verification Date	: 21/01/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 46 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd-El-Aty.
Dr. Mennan Hossam.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Name : Rsha Ramadan Ahmed	**Date : 09-02-2026 11:09**
Branch : Portable	**Medical No. : GRI-27907250102485**
Ref.Physician Prof : CAP+C	**Ins.Company :**

- Normal C.T appearance of the spleen (splenule seen posteriorly measures about 1x0.7 cm and smaller one seen anterior measures about 0.8x0.6 cm), pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboliths
- No ascites.
- **The examined skeleton** D10, L2 small lytic areas **for further bone scan.**

Opinion:

- **Bilateral few scattered subpleural pulmonary nodules.**
- **Hepatomegaly with two small hepatic focal lesions for further triphasic CT/ dynamic MRI.**
- **D10, L2 small lytic areas for further bone scan**

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL

Name : Rsha Ramadan Ahmed
Branch : Portable
Ref.Physician Prof : CAP+C

Date : 09-02-2026 11:09
Medical No. : GRI-27907250102485
Ins.Company :

CT Scan
[[DOCTYPE:CT_SCAN]]

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of right breast cancer for metastatic workup.

I- CHEST:

- The right breast showed UIQ soft tissue mass lesion measures about 7.5x5 cm in its TS and AP dimensions respectively. It showed central necrotic area and seen abutting the underlying pectoralis muscle. Associated with multiple enlarged ipsilateral axillary lymph nodes, the largest is seen measuring about 18x10mm. **for further sono-mammography.**
- Azygos lobe is noted
- Few bilateral scattered subpleural pulmonary nodules are noted, the largest is seen in right lower lobar posterior segment measuring about 5mm.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- The liver is enlarged in size showing homogenous parenchymal attenuation. The left hepatic lobe segment IVa showed well defined hypodense lesion in postcontrast study measures about 6.5x5 mm, another smaller one is seen in the right hepatic lobe segment V measures about 5.4x4.5mm **for further triphasic CT/ dynamic MRI.**
- No dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.

Name: Mrs. Rasha Ramadan Ahmed
Date: 2026-01-25

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven right breast IDC, grade III, triple positive molecular subtype, KI 67: 70 %, re-assessment study.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- UIQ, middle third, irregular partially speculated partially lobulated high-density mass lesion with surrounding distortion and suspicious pleomorphic clusters of microcalcifications.
- Ultrasound demonstrated 1& 2 o'clock, middle third, 3.5 cm from nipple, 3 mm deep to the skin, irregular spiculated heterogenous hypoechoic solid mass lesion with microcalcifications within & surrounding echogenic halo, posterior acoustic shadowing as well as moderate internal vascularity on color Doppler study measuring 6.5 x 4.2 cm.
- Overlying focal skin thickness is noted reaching 5 mm.
- Prominent related ducts with echogenic content is seen reaching the nipple.
- Multiple (more than three) discrete pathological level 1 axillary LNs are noted, the largest measured 15 x 12 mm while one showed irregular outline ?? extra-capsular extension measuring 12 x 9 mm. multiple pathological nodes are noted at levels 2 & 3. One internal mammary lymph node is noted at the 1st intercostal space measuring 16 x 7 mm with 5 mm cortical thickness. No pathological supraclavicular lymph nodes.

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion: -

Right breast (BIRADS 6): locally advanced malignant process with ipsilateral pathological levels 1, 2, 3 and internal mammary lymph nodes.

Left breast (BIRADS 1): Normal study.

Recommendation:-

For CESM.

Best Regards,
DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name : Rasha Ramadan Ahmed Aleem
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 04-02-2026 12:00
Medical No. : 309577
Ins.Company : تعاقد مستشفيات

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 46-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]